

ATPL EASA STUDY GUIDE | 040 - HUMAN PERFORMANCE

Chapter 6: Hearing & Vestibular System

Syllabus Source: CAE Oxford (pp. 159-204) | Pass Target: >= 90% AviationExam

1. ANATOMY OF THE EAR: HEARING & EQUILIBRIUM

The human ear performs two completely distinct physiological functions: acoustic perception (hearing) and vestibular sensing (spatial orientation and equilibrium):

Anatomical Region	Key Structures	Physiological Function
Outer Ear	Pinna & External Canal	Collects sound waves and funnels them to the tympanic membrane (eardrum).
Middle Ear	Ossicles (Hammer, Anvil, Stirrup)	Air-filled cavity. Amplifies mechanical sound vibrations (20:1 ratio) to oval window. Equalized by Eustachian tube.
Inner Ear (Cochlea)	Organ of Corti	Fluid-filled snail-like structure containing hair cells that convert hydraulic pressure waves into neural impulses to cranial nerve VIII.
Inner Ear (Vestibular)	Semicircular Canals & Otoliths	Detects angular acceleration (canals) and linear acceleration / gravity (otoliths).

2. THE VESTIBULAR APPARATUS (CANALS VS OTOLITHS)

The vestibular system is designed exclusively for a terrestrial environment (1G gravity). In three-dimensional flight, acceleration forces constantly deceive its sensory mechanisms:

Vestibular Organ	Sensed Movement Type	Biophysical Mechanism & Limitations
Semicircular Canals (3 orthogonal planes: roll, pitch, yaw)	Angular Acceleration (Rotational)	Endolymph fluid deflects the gelatinous CUPULA. Perception threshold: 0.5° to 2.0°/s ² . In a constant-rate turn, endolymph stabilizes in 10-20 seconds, and the brain senses NO TURN!
Otolith Organs (Utricle & Sacculle)	Linear Acceleration & Gravity	Contains OTOLITHS (calcium carbonate stones) on a sensory gel bed. Linear acceleration pulls otoliths rearward, creating an identical sensory signal to head pitch up!

3. MASTER FLIGHT VESTIBULAR ILLUSIONS (SPATIAL DISORIENTATION)

Vestibular Illusion	Triggering Flight Condition	Physiological Mechanism & Flight Danger
The Leans	Most common illusion	An unnoticed roll (< 2°/s) is followed by a rapid wings-level correction. The pilot feels a strong roll in the opposite direction and leans body into turn.
Somatogravic Illusion	Rapid acceleration on take-off / go-around	Inertia displaces otoliths backward. The brain perceives a steep NOSE-UP pitch. The pilot pushes the stick forward into the ground (CFIT risk).

Graveyard Spin	Prolonged spin recovery	After 15-20 s of spinning, canals adapt (feeling stops). Applying opposite rudder to recover deflects cupula, creating the illusion of spinning in the opposite direction; pilot re-enters original spin!
Graveyard Spiral	Descending coordinated spiral	Canals adapt to turn. Pilot senses wings level, sees altitude loss on altimeter, pulls back on elevator, tightening the spiral dive!
Coriolis Illusion	Head movement in steady turn	Tilting head in a different plane during a turn stimulates two canals simultaneously, creating a violent, tumbling sensation of uncontrolled pitch/roll/yaw.

[!] EXAM TRAP (AVIATIONEXAM) - The Golden Rule for Overcoming Spatial Disorientation

When experiencing spatial disorientation or conflict between bodily sensations and flight instruments: 1. NEVER TRUST YOUR 'SEAT-OF-THE-PANTS' SENSATIONS! 2. BELIEVE YOUR FLIGHT INSTRUMENTS 100% (Attitude Indicator, Altimeter, VSI, Turn Coordinator); 3. Suppress all head movements (avoids Coriolis illusion); 4. If available, transfer aircraft control to the other pilot or engage the autopilot.

4. NOISE EXPOSURE, HEARING LOSS & MOTION SICKNESS

- Decibel Scale (Logarithmic): Every 3 dB increase DOUBLES sound energy. Permissible continuous exposure without protection is 85 dB for 8 hours (88 dB for 4h, 91 dB for 2h). Pain threshold: 140 dB.
- Sensorineural Hearing Loss (NIHL): Irreversible destruction of cochlear hair cells caused by chronic noise exposure. Characterized by a distinctive acoustic notch at 4,000 Hz!
- Motion Sickness (Kinetosis): Triggered by sensory conflict (mismatch) between visual, vestibular, and proprioceptive inputs (e.g. eyes see static cockpit while inner ear senses turbulent roll/pitch).

5. AVIATIONEXAM PRACTICE QUESTIONS (WITH EXPLANATIONS)

Q1: What is the primary cause of the Somatogravic Illusion during a night or instrument take-off?

[A] Semicircular canal fluid drying out [B] Linear acceleration displacing otoliths backward, which the brain interprets as a steep pitch-up [C] Rapid change in cabin pressure on climb [D] High positive G-forces draining blood from the brain

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [B]. Forward linear acceleration moves otolithic stones backward, identical to backward head tilt (pitch up). The pilot falsely pushes nose down.

Q2: In a prolonged coordinated turn lasting more than 20 seconds, what sensation do the semicircular canals t

[A] A violent tumbling sensation [B] A feeling of constant roll at 45° [C] Sensation that the turn has stopped and the aircraft is flying straight and level [D] Severe vertigo

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [C]. After 10 to 20 seconds of constant angular velocity, endolymph fluid matches canal rotation speed; the cupula returns to neutral (sensing zero turn).

Q3: Which sensory illusion is triggered by moving the head out of the plane of rotation while the aircraft is in a turn?

[A] Somatogravic illusion [B] Coriolis illusion [C] Autokinesis [D] The Leans

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [B]. The Coriolis illusion occurs when head movement stimulates two semicircular canals simultaneously during a turn, creating an overwhelming tumbling sensation.

Q4: What is 'The Leans' in aviation spatial disorientation?

[A] An otolith illusion occurring only during landing flare [B] A false sensation of roll following an undetected slow roll and rapid wings-level correction, prompting the pilot to lean [C] Severe neck muscle spasms caused by G-forces [D] A vestibular symptom of carbon monoxide poisoning

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [B]. Rolling slowly below threshold ($< 2^\circ/\text{s}$) and abruptly returning to level stimulates the cupula in reverse, making the pilot feel tilted and lean to compensate.

Q5: Noise-induced hearing loss (NIHL) is characterized by an initial hearing deficit at which frequency?

[A] 500 Hz [B] 1,000 Hz [C] 4,000 Hz [D] 12,000 Hz

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [C]. Acoustic trauma and chronic noise exposure characteristically damage cochlear hair cells at the 4,000 Hz notch first.

Q6: What is the most effective immediate countermeasure when a pilot experiences spatial disorientation in IMC?

[A] Close eyes and shake head vigorously [B] Disregard bodily sensations completely and fly strictly by reference to the primary flight instruments [C] Look out of the window for ground references [D] Perform a 360° turn to reset the vestibular canals

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [B]. Spatial disorientation is overcome solely by suppressing vestibular cues and strictly believing the flight instruments.

Q7: What is the underlying physiological mechanism of motion sickness (kinetosis) in flight?

[A] Severe hypoxia in the cochlea [B] A sensory mismatch between visual cues, vestibular signals, and proprioceptive receptors [C] Sudden decrease in blood sugar level [D] Decompression of the middle ear cavity

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [B]. The sensory mismatch theory explains motion sickness as an evolutionary defense triggered when the brain receives contradictory movement signals from eyes and ears.

Q8: Why are the Otolith Organs (Utricle and Saccule) physically incapable of distinguishing between linear acceleration and constant velocity?

[A] They lack blood supply during acceleration [B] Inertial force from forward acceleration displaces the otoliths backwards in the exact same manner as gravitational pull during head pitch-up [C] They only respond to sound waves above 1,000 Hz [D] They are deactivated above 10,000 ft

[*] OFFICIAL DEFINITION - Detailed Answer Explanation

CORRECT: [B]. Forward acceleration creates an inertial reaction vector that shifts the otolith mass backward, perfectly mimicking a nose-up attitude.